

DUROSAND K5

Durotech Industries

Chemwatch: 5258-39

Version No: 3.1.1.1

Safety Data Sheet according to WHS and ADG requirements

Chemwatch Hazard Alert Code: 4

Issue Date: 30/08/2017

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L.GHS.AUS.EN

SECTION 1 IDENTIFICATION OF THE SUBSTANCE / MIXTURE AND OF THE COMPANY / UNDERTAKING

Product Identifier

Product name	DUROSAND K5
Synonyms	filler
Other means of identification	Not Available

Relevant identified uses of the substance or mixture and uses advised against

Relevant identified uses	Use according to manufacturer's directions. Filler.
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Details of the supplier of the safety data sheet

Registered company name	Durotech Industries
Address	14 Essex Street Minto NSW 2566 Australia
Telephone	02 9603 1177
Fax	02 9475 5059
Website	www.durotechindustries.com.au
Email	accounts@durotechindustries.com.au

Emergency telephone number

Association / Organisation	Not Available
Emergency telephone numbers	0421 670 636
Other emergency telephone numbers	Not Available

SECTION 2 HAZARDS IDENTIFICATION

Classification of the substance or mixture

HAZARDOUS CHEMICAL. NON-DANGEROUS GOODS. According to the WHS Regulations and the ADG Code.

Poisons Schedule	Not Applicable
Classification ^[1]	Carcinogenicity Category 2, Specific target organ toxicity - repeated exposure Category 2, Acute Aquatic Hazard Category 3
Legend:	1. Classified by Chemwatch; 2. Classification drawn from HSIS ; 3. Classification drawn from EC Directive 1272/2008 - Annex VI

Label elements

Hazard pictogram(s)	
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SIGNAL WORD	WARNING
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Hazard statement(s)

H351	Suspected of causing cancer.
H373	May cause damage to organs through prolonged or repeated exposure.
H402	Harmful to aquatic life

Precautionary statement(s) Prevention

P201	Obtain special instructions before use.
P260	Do not breathe dust/fume/gas/mist/vapours/spray.
P281	Use personal protective equipment as required.

P273	Avoid release to the environment.
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Precautionary statement(s) Response

P308+P313	IF exposed or concerned: Get medical advice/attention.
P314	Get medical advice/attention if you feel unwell.

Precautionary statement(s) Storage

P405	Store locked up.
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Precautionary statement(s) Disposal

P501	Dispose of contents/container in accordance with local regulations.
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SECTION 3 COMPOSITION / INFORMATION ON INGREDIENTS**Substances**

See section below for composition of Mixtures

Mixtures

CAS No	%[weight]	Name
14808-60-7	>60	<u>silica crystalline - quartz</u>
68515-48-0	<0.5	<u>diisononyl phthalate</u>
65997-17-3	1-10	<u>glass fibres</u>

SECTION 4 FIRST AID MEASURES**Description of first aid measures**

Eye Contact	If this product comes in contact with the eyes: - Wash out immediately with fresh running water. - Ensure complete irrigation of the eye by keeping eyelids apart and away from eye and moving the eyelids by occasionally lifting the upper and lower lids. - Seek medical attention without delay; if pain persists or recurs seek medical attention. - Removal of contact lenses after an eye injury should only be undertaken by skilled personnel.
Skin Contact	If skin or hair contact occurs: - Flush skin and hair with running water (and soap if available). - Seek medical attention in event of irritation.
Inhalation	- If fumes or combustion products are inhaled remove from contaminated area. - Lay patient down. Keep warm and rested. - Prostheses such as false teeth, which may block airway, should be removed, where possible, prior to initiating first aid procedures. - Apply artificial respiration if not breathing, preferably with a demand valve resuscitator, bag-valve mask device, or pocket mask as trained. Perform CPR if necessary. - Transport to hospital, or doctor.
Ingestion	If swallowed do NOT induce vomiting. - If vomiting occurs, lean patient forward or place on left side (head-down position, if possible) to maintain open airway and prevent aspiration. - Observe the patient carefully. - Never give liquid to a person showing signs of being sleepy or with reduced awareness; i.e. becoming unconscious. - Give water to rinse out mouth, then provide liquid slowly and as much as casualty can comfortably drink. - Seek medical advice.

Indication of any immediate medical attention and special treatment needed

Treat symptomatically.

SECTION 5 FIREFIGHTING MEASURES**Extinguishing media**

- There is no restriction on the type of extinguisher which may be used.
- Use extinguishing media suitable for surrounding area.

Special hazards arising from the substrate or mixture

Fire Incompatibility	None known.
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Advice for firefighters

Fire Fighting	- When silica dust is dispersed in air, firefighters should wear inhalation protection as hazardous substances from the fire may be adsorbed on the silica particles. - When heated to extreme temperatures, (>1700 deg.C) amorphous silica can fuse. - Alert Fire Brigade and tell them location and nature of hazard. - Wear breathing apparatus plus protective gloves in the event of a fire. - Prevent, by any means available, spillage from entering drains or water courses. - Use fire fighting procedures suitable for surrounding area. DO NOT approach containers suspected to be hot. - Cool fire exposed containers with water spray from a protected location. - If safe to do so, remove containers from path of fire. - Equipment should be thoroughly decontaminated after use.
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Fire/Explosion Hazard	▶ Non combustible. ▶ Not considered a significant fire risk, however containers may burn. Decomposition may produce toxic fumes of: silicon dioxide (SiO₂) May emit poisonous fumes. May emit corrosive fumes.
HAZCHEM	Not Applicable

SECTION 6 ACCIDENTAL RELEASE MEASURES

Personal precautions, protective equipment and emergency procedures

See section 8

Environmental precautions

See section 12

Methods and material for containment and cleaning up

Minor Spills	▶ Clean up waste regularly and abnormal spills immediately. ▶ Avoid breathing dust and contact with skin and eyes. ▶ Wear protective clothing, gloves, safety glasses and dust respirator. ▶ Use dry clean up procedures and avoid generating dust. ▶ Vacuum up or sweep up. NOTE: Vacuum cleaner must be fitted with an exhaust micro filter (HEPA type) (consider explosion-proof machines designed to be grounded during storage and use). ▶ Dampen with water to prevent dusting before sweeping. ▶ Place in suitable containers for disposal.
Major Spills	Moderate hazard. ▶ CAUTION: Advise personnel in area. ▶ Alert Emergency Services and tell them location and nature of hazard. ▶ Control personal contact by wearing protective clothing. ▶ Prevent, by any means available, spillage from entering drains or water courses. ▶ Recover product wherever possible. ▶ IF DRY: Use dry clean up procedures and avoid generating dust. Collect residues and place in sealed plastic bags or other containers for disposal. IF WET: Vacuum/shovel up and place in labelled containers for disposal. ▶ ALWAYS: Wash area down with large amounts of water and prevent runoff into drains. ▶ If contamination of drains or waterways occurs, advise Emergency Services.

Personal Protective Equipment advice is contained in Section 8 of the SDS.

SECTION 7 HANDLING AND STORAGE

Precautions for safe handling

Safe handling	▶ The use of ceramic fibres in the work place should be reviewed in the context of frequency of use and potential for exposure. ▶ In circumstances where the respiratory standards or excursion limits are approached, work areas should be designated by the use of ropes or other similar barriers and appropriate signs be utilised, where possible. This is especially true for all overhead work involving ceramic fibres. ▶ Employees not engaged in the ceramic fibre work should not be allowed within 3 metres of the work unless wearing suitable personal protective equipment (PPE). ▶ An example of the appropriate signage for the restricted area is: CERAMIC FIBRE WORK AREA; FOLLOW SAFETY INSTRUCTIONS. All installation and/ or removal practices should be designed to minimise the liberation of dusts or fibres. For Installation: ▶ The ceramic fibre material should be kept in its storage container until installation is ready to proceed. ▶ Containers/ bags should only be opened within the designated work areas. ▶ Empty storage bags should be stored in waste containers along with waste material. For Removal: ▶ Waste material should be wetted to prevent generation of dusts and placed in sealed containers to prevent dust/ fibre emissions. Upon completion of installation/ removal: ▶ All excess material should be sealed in bags/ containers prior to removal from designated work area. ▶ Area should then be cleaned using an industrial vacuum cleaner. ▶ Any remaining contaminant material should be removed with minimum liberation of dusts/fibres. ▶ Wet mopping and wiping may be utilised in some instances when an industrial vacuum is not available. ▶ Avoid all personal contact, including inhalation. ▶ Wear protective clothing when risk of exposure occurs. ▶ Use in a well-ventilated area. ▶ Prevent concentration in hollows and sumps. ▶ DO NOT enter confined spaces until atmosphere has been checked. ▶ DO NOT allow material to contact humans, exposed food or food utensils. ▶ Avoid contact with incompatible materials. ▶ When handling, DO NOT eat, drink or smoke. ▶ Keep containers securely sealed when not in use. ▶ Avoid physical damage to containers. ▶ Always wash hands with soap and water after handling. ▶ Work clothes should be laundered separately. Launder contaminated clothing before re-use. ▶ Use good occupational work practice. ▶ Observe manufacturer's storage and handling recommendations contained within this SDS. ▶ Atmosphere should be regularly checked against established exposure standards to ensure safe working conditions are maintained.
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Other information

- ▶ Store in original containers.
- ▶ Keep containers securely sealed.
- ▶ Store in a cool, dry area protected from environmental extremes.
- ▶ Store away from incompatible materials and foodstuff containers.
- ▶ Protect containers against physical damage and check regularly for leaks.
- ▶ Observe manufacturer's storage and handling recommendations contained within this SDS.

For major quantities:

- ▶ Consider storage in banded areas - ensure storage areas are isolated from sources of community water (including stormwater, ground water, lakes and streams).
- ▶ Ensure that accidental discharge to air or water is the subject of a contingency disaster management plan; this may require consultation with local authorities.

Conditions for safe storage, including any incompatibilities

Suitable container

- ▶ Polyethylene or polypropylene container.
- ▶ Check all containers are clearly labelled and free from leaks.

Storage incompatibility

Phthalates:

- ▶ react with strong acids, strong oxidisers, permanganates and nitrates
- ▶ attack some form of plastics

Silicas:

- ▶ react with hydrofluoric acid to produce silicon tetrafluoride gas
- ▶ react with xenon hexafluoride to produce explosive xenon trioxide
- ▶ reacts exothermically with oxygen difluoride, and explosively with chlorine trifluoride (these halogenated materials are not commonplace industrial materials) and other fluorine-containing compounds
- ▶ may react with fluorine, chlorates
- ▶ are incompatible with strong oxidisers, manganese trioxide, chlorine trioxide, strong alkalis, metal oxides, concentrated orthophosphoric acid, vinyl acetate
- ▶ may react vigorously when heated with alkali carbonates.
- ▶ Metals and their oxides or salts may react violently with chlorine trifluoride and bromine trifluoride.
- ▶ These trifluorides are hypergolic oxidisers. They ignite on contact (without external source of heat or ignition) with recognised fuels - contact with these materials, following an ambient or slightly elevated temperature, is often violent and may produce ignition.
- ▶ The state of subdivision may affect the results.

SECTION 8 EXPOSURE CONTROLS / PERSONAL PROTECTION

Control parameters

OCCUPATIONAL EXPOSURE LIMITS (OEL)

INGREDIENT DATA

Source	Ingredient	Material name	TWA	STEL	Peak	Notes
Australia Exposure Standards	silica crystalline - quartz	Silica - Crystalline	Not Available	Not Available	Not Available	Not Available
Australia Exposure Standards	silica crystalline - quartz	Quartz (respirable dust)	0.1 mg/m3	Not Available	Not Available	Not Available
Australia Exposure Standards	silica crystalline - quartz	Quartz (respirable dust)	0.1 mg/m3	Not Available	Not Available	Not Available
Australia Exposure Standards	glass fibres	Man-Made Vitreous (Silicate) Fibres (MMVF)	Not Available	Not Available	Not Available	Not Available
Australia Exposure Standards	glass fibres	Synthetic mineral fibres (SMF)	Not Available	Not Available	Not Available	Not Available

EMERGENCY LIMITS

Ingredient	Material name	TEEL-1	TEEL-2	TEEL-3
silica crystalline - quartz	Silica, crystalline-quartz; (Silicon dioxide)	0.075 mg/m3	33 mg/m3	200 mg/m3
glass fibres	Fibrous glass; (Fiber glass; Glass frit; Synthetic vitreous fibers)	15 mg/m3	170 mg/m3	990 mg/m3

Ingredient	Original IDLH	Revised IDLH
silica crystalline - quartz	N.E. mg/m3 / N.E. ppm	50 mg/m3
diisononyl phthalate	Not Available	Not Available
glass fibres	Not Available	Not Available

MATERIAL DATA

WARNING: For inhalation exposure ONLY: This substance has been classified by the IARC as Group 1: **CARCINOGENIC TO HUMANS**

The International Agency for Research on Cancer (IARC) has classified occupational exposures to **respirable** (<5 µm) crystalline silica as being carcinogenic to humans. This classification is based on what IARC considered sufficient evidence from epidemiological studies of humans for the carcinogenicity of inhaled silica in the forms of quartz and cristobalite. Crystalline silica is also known to cause silicosis, a non-cancerous lung disease.

Intermittent exposure produces; focal fibrosis, (pneumoconiosis), cough, dyspnoea, liver tumours.

* Millions of particles per cubic foot (based on impinger samples counted by light field techniques).

NOTE : the physical nature of quartz in the product determines whether it is likely to present a chronic health problem. To be a hazard the material must enter the breathing zone as respirable particles.

Because the margin of safety of the quartz TLV is not known with certainty and given the associated link between silicosis and lung cancer it is recommended that quartz concentrations be maintained as far below the TLV as prudent practices will allow.

Exposure to respirable crystalline silicas (RCS) represents a significant hazard to workers, particularly those employed in the construction industry where respirable dusts of cement and concrete are common. Cutting, grinding and other high speed processes, involving their finished products, may further result in dusty atmospheres. Bricks are also a potential source of RCSs under such circumstances.

It is estimated that half of the occupations, involved in construction work, are exposed to levels of RCSs, higher than the current allowable limits. Beaudry et al: Journal of Occupational and Environmental Hygiene 10: 71-77; 2013

for glass wool fibres, rock wool fibres, slag wool fibres, and special purpose glass fibres

Respirable fibres:

TLV TWA: 1 fibre/cc, A3 (respirable fibres, length >5 µm, aspect ratio ≥ 3:1 as determined by the membrane filter method at 400-450X magnification (4 mm objective) phase contrast illumination).

CAUTION: This substance has been classified by the ACGIH as A3; Animal Carcinogen (at relatively high doses).

Continued...

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ES TWA: 0.5 fibres/ml [NOHSC, Australia]

Exposure Levels to be determined in accordance with NOHSC: 3006; Guidance Note on Membrane Filter Method for Estimating Airborne SMF

Earlier FARIMA/ACTU action plan recommends the following:

Action level 0.2 fibre/ml (respirable dust)

<0.2 fibres/ml: Basic OH&S hygiene procedures

0.2-0.5 fibres/ml: Control strategies implemented and personal protection provided.

>0.5 fibres/ml: Immediate steps should be taken to reduce this level.

Extensive monitoring history indicates that airborne respirable fibre levels will rarely exceed 0.2 fibres/ml in most user applications, provided the handling and storage of fibres conforms with the recommendations made elsewhere in this document.

Inspirable dusts:

ES TWA: 2 mg/m³ inspirable dust (non-respirable fibres) (ASCC proposal as a second level standard); mineral wool fibre, containing no asbestos and < 1% free silica

TLV TWA: 10 mg/m³, total dust [ACGIH]

In a criteria document on fibrous glass NIOSH established a REL-TWA of 3 fibres/cc (fibre less than or equal to 3.5 µm in diameter and greater than or equal to 10 µm in length) and 5 mg/m³ (total mineral wool). It also stated that an exposure limit of 10 mg/m³ is highly unlikely to be protective for exposed employees.

NOTE: exposure standards described here do not apply continuous filament glass fibres and refractory ceramic fibres.

No exposure standards currently exists for biosoluble respirable fibres; in their absence it would be prudent to adopt the standards cited here.

Synthetic vitreous fibres are composed largely of aluminium and calcium silicates derived from rock, clay, slag or glass. For the purpose of classification they are divided into two broad classes: filaments and wools. Filaments contain continuous glass filaments, while wools contain glass wool, rock (stone) wool, slag wool, refractory ceramic fibres and other newly engineered biosoluble fibres. Generally wool fibres tend to be shorted and finer than continuous filament fibres and their diameters more variable.

All fibrous glass products consist of silicon and aluminium oxides. The final properties of the glass are dictated by the percent composition of other oxides including alkali metal oxides (e.g. Na₂O, K₂O), alkaline earth oxides (e.g. CaO, MgO) and metal oxides (e.g. ZrO₂, Fe₂O₃). The term "mineral wool" is used to describe rock wool and slag wool and occasionally glass wool. Refractory ceramic fibres (RCFs) are a specialised type of synthetic vitreous fibre that are highly heat resistant and contain a much higher concentration of alumina (Al₂O₃) than other fibres. Although RCFs are amorphous at low temperatures, they undergo partial crystallisation (devitrification) to quartz, cristobolite, or tridymite at the elevated temperatures for which they were designed.

The diameter of airborne fibres are an important physical property from a biological standpoint because thin fibres are considered respirable and may be deposited in the peripheral lung airway.

Airborne fibres with diameters < 3 µm are generally considered respirable in humans. There is a strong correlation between fibre diameter and airborne fibre levels found in the work place.

Generally the smaller the fibre diameter, the lower the airborne fibre concentration. Rock wool and slag wool typically possess fibre diameters in the range of about 3-7 µm. Glass wool possesses

fibre diameters typically of 3-15 µm. The smaller diameters of these fibres in comparison to continuous filament fibres allows for the possibility that a small fraction of these fibres may become

respirable when they become airborne. Special purpose glass fibres typically have fibre diameters of <3 µm and often < 1 µm. RCFs typically have fibre diameters of 1-5 µm

Exposure standards for man-made vitreous (silicate) fibres (also known as MMMF, SMF or MMVF) are currently based on a combination of chemical and physical properties.

In general they fall into one of two groups:

Those with random orientation with alkaline oxide and alkaline earth oxide (Na₂O+K₂O +CaO+MgO+BaO) content:

- greater than 18% by weight (generally the mineral wools)
- less than 18% weight: this group (Refractory Ceramic Fibres (RCF) and special purpose fibres).

Members of both groups may produce cancer but certain fibre properties must be considered before assigning this classification These are described in Notes Q, and R of Annex 1 of the European Directive 67/548/EEC.

Note Q - this applies to the first group (the mineral wools) only:

The classification as a carcinogen need not apply if it can be shown that the substance fulfils one of the following conditions:

- a short term biopersistence test by inhalation has shown that the fibres longer than 20 µm have a weighted half-life less than 10 days (the so-called biosoluble fibres), or
- a short term biopersistence test by intratracheal instillation has shown that the fibres longer than 20 µm have a weighted half-life less than 40 days (biosoluble fibres), or
- an appropriate intra-peritoneal test has shown no evidence of excess carcinogenicity, or
- absence of relevant pathogenicity or neoplastic changes in a suitable long term inhalation test.

Note R - this applies to both groups (the mineral wools and RCFs)

The classification as a carcinogen need not apply to fibres with a length weighted geometric mean diameter less two standard geometric errors greater than 6 µm.

The ACGIH has created exposure standards for each group:

For mineral wools (glass wool, rock wool and slag wool) a TWA of 1 f/cc pertains

For RCFs (and special purpose fibres) a TWA of 0.2 f/cc pertains

Biosoluble fibres(described in Note Q) have not yet been assigned a value - because they may create irritation, however, a "Nuisance Particulate" value cannot apply. Although not actually soluble in water the term dissolution is often used to describe the durability of synthetic vitreous fibres, especially as it pertains to biological fluid. Dissolution produces degradation. Under alkaline and acidic conditions, the silicate network of synthetic vitreous fibres can be attacked, resulting in leaching of individual ions and the eventual disruption of the entire fibre network.

The concentration of dust, for application of respirable dust limits, is to be determined from the fraction that penetrates a separator whose size collection efficiency is described by a cumulative log-normal function with a median aerodynamic diameter of 4.0 µm (+-) 0.3 µm and with a geometric standard deviation of 1.5 µm (+-) 0.1 µm, i.e.,generally less than 5 µm.

Exposure controls

Appropriate engineering controls

Engineering controls are used to remove a hazard or place a barrier between the worker and the hazard. Well-designed engineering controls can be highly effective in protecting workers and will typically be independent of worker interactions to provide this high level of protection.

The basic types of engineering controls are:

Process controls which involve changing the way a job activity or process is done to reduce the risk.

Enclosure and/or isolation of emission source which keeps a selected hazard "physically" away from the worker and ventilation that strategically "adds" and "removes" air in the work environment. Ventilation can remove or dilute an air contaminant if designed properly. The design of a ventilation system must match the particular process and chemical or contaminant in use.

Employers may need to use multiple types of controls to prevent employee overexposure.

Local exhaust ventilation usually required. If risk of overexposure exists, wear approved respirator. Correct fit is essential to obtain adequate protection.

Supplied-air type respirator may be required in special circumstances. Correct fit is essential to ensure adequate protection.

An approved self contained breathing apparatus (SCBA) may be required in some situations.

Provide adequate ventilation in warehouse or closed storage area. Air contaminants generated in the workplace possess varying "escape" velocities which, in turn, determine the "capture velocities" of fresh circulating air required to effectively remove the contaminant.

Type of Contaminant:	Air Speed:
solvent, vapours, degreasing etc., evaporating from tank (in still air).	0.25-0.5 m/s (50-100 f/min.)
aerosols, fumes from pouring operations, intermittent container filling, low speed conveyer transfers, welding, spray drift, plating acid fumes, pickling (released at low velocity into zone of active generation)	0.5-1 m/s (100-200 f/min.)
direct spray, spray painting in shallow booths, drum filling, conveyer loading, crusher dusts, gas discharge (active generation into zone of rapid air motion)	1-2.5 m/s (200-500 f/min.)
grinding, abrasive blasting, tumbling, high speed wheel generated dusts (released at high initial velocity into zone of very high rapid air motion).	2.5-10 m/s (500-2000 f/min.)

Within each range the appropriate value depends on:

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	<table> <tr> <th>Lower end of the range</th><th>Upper end of the range</th></tr> <tr> <td>1: Room air currents minimal or favourable to capture</td><td>1: Disturbing room air currents</td></tr> <tr> <td>2: Contaminants of low toxicity or of nuisance value only.</td><td>2: Contaminants of high toxicity</td></tr> <tr> <td>3: Intermittent, low production.</td><td>3: High production, heavy use</td></tr> <tr> <td>4: Large hood or large air mass in motion</td><td>4: Small hood-local control only</td></tr> </table> Simple theory shows that air velocity falls rapidly with distance away from the opening of a simple extraction pipe. Velocity generally decreases with the square of distance from the extraction point (in simple cases). Therefore the air speed at the extraction point should be adjusted, accordingly, after reference to distance from the contaminating source. The air velocity at the extraction fan, for example, should be a minimum of 1-2 m/s (200-400 f/min) for extraction of solvents generated in a tank 2 meters distant from the extraction point. Other mechanical considerations, producing performance deficits within the extraction apparatus, make it essential that theoretical air velocities are multiplied by factors of 10 or more when extraction systems are installed or used.	Lower end of the range	Upper end of the range	1: Room air currents minimal or favourable to capture	1: Disturbing room air currents	2: Contaminants of low toxicity or of nuisance value only.	2: Contaminants of high toxicity	3: Intermittent, low production.	3: High production, heavy use	4: Large hood or large air mass in motion	4: Small hood-local control only
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Personal protection											
Eye and face protection	▶ Safety glasses with side shields. ▶ Chemical goggles. ▶ Contact lenses may pose a special hazard; soft contact lenses may absorb and concentrate irritants. A written policy document, describing the wearing of lenses or restrictions on use, should be created for each workplace or task. This should include a review of lens absorption and adsorption for the class of chemicals in use and an account of injury experience. Medical and first-aid personnel should be trained in their removal and suitable equipment should be readily available. In the event of chemical exposure, begin eye irrigation immediately and remove contact lens as soon as practicable. Lens should be removed at the first signs of eye redness or irritation - lens should be removed in a clean environment only after workers have washed hands thoroughly. [CDC NIOSH Current Intelligence Bulletin 59], [AS/NZS 1336 or national equivalent]										
Skin protection	See Hand protection below										
Hands/feet protection	The selection of suitable gloves does not only depend on the material, but also on further marks of quality which vary from manufacturer to manufacturer. Where the chemical is a preparation of several substances, the resistance of the glove material can not be calculated in advance and has therefore to be checked prior to the application. The exact break through time for substances has to be obtained from the manufacturer of the protective gloves and has to be observed when making a final choice. Personal hygiene is a key element of effective hand care. Gloves must only be worn on clean hands. After using gloves, hands should be washed and dried thoroughly. Application of a non-perfumed moisturizer is recommended. Suitability and durability of glove type is dependent on usage. Important factors in the selection of gloves include: - frequency and duration of contact, - chemical resistance of glove material, - glove thickness and - dexterity Select gloves tested to a relevant standard (e.g. Europe EN 374, US F739, AS/NZS 2161.1 or national equivalent). - When prolonged or frequently repeated contact may occur, a glove with a protection class of 5 or higher (breakthrough time greater than 240 minutes according to EN 374, AS/NZS 2161.10.1 or national equivalent) is recommended. - When only brief contact is expected, a glove with a protection class of 3 or higher (breakthrough time greater than 60 minutes according to EN 374, AS/NZS 2161.10.1 or national equivalent) is recommended. - Some glove polymer types are less affected by movement and this should be taken into account when considering gloves for long-term use. - Contaminated gloves should be replaced. For general applications, gloves with a thickness typically greater than 0.35 mm, are recommended. It should be emphasised that glove thickness is not necessarily a good predictor of glove resistance to a specific chemical, as the permeation efficiency of the glove will be dependent on the exact composition of the glove material. Therefore, glove selection should also be based on consideration of the task requirements and knowledge of breakthrough times. Glove thickness may also vary depending on the glove manufacturer, the glove type and the glove model. Therefore, the manufacturers' technical data should always be taken into account to ensure selection of the most appropriate glove for the task. Note: Depending on the activity being conducted, gloves of varying thickness may be required for specific tasks. For example: - Thinner gloves (down to 0.1 mm or less) may be required where a high degree of manual dexterity is needed. However, these gloves are only likely to give short duration protection and would normally be just for single use applications, then disposed of. - Thicker gloves (up to 3 mm or more) may be required where there is a mechanical (as well as a chemical) risk i.e. where there is abrasion or puncture potential Gloves must only be worn on clean hands. After using gloves, hands should be washed and dried thoroughly. Application of a non-perfumed moisturiser is recommended. Experience indicates that the following polymers are suitable as glove materials for protection against undissolved, dry solids, where abrasive particles are not present. ▶ polychloroprene. ▶ nitrile rubber. ▶ butyl rubber. ▶ fluorocautchouc. ▶ polyvinyl chloride. Gloves should be examined for wear and/ or degradation constantly.										
Body protection	See Other protection below										
Other protection	▶ Overalls. ▶ P.V.C. apron. ▶ Barrier cream. ▶ Skin cleansing cream. ▶ Eye wash unit.										
Thermal hazards	Not Available										

Respiratory protection

Type A-P Filter of sufficient capacity. (AS/NZS 1716 & 1715, EN 143:2000 & 149:2001, ANSI Z88 or national equivalent)

Required Minimum Protection Factor	Half-Face Respirator	Full-Face Respirator	Powered Air Respirator
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up to 10 x ES	A P1 Air-line*	-	A PAPR-P1
up to 50 x ES	Air-line**	A P2	A PAPR-P2
up to 100 x ES	-	A P3	-
		Air-line*	-
100+ x ES	-	Air-line**	A PAPR-P3

* - Negative pressure demand ** - Continuous flow

A(All classes) = Organic vapours, B AUS or B1 = Acid gasses, B2 = Acid gas or hydrogen cyanide(HCN), B3 = Acid gas or hydrogen cyanide(HCN), E = Sulfur dioxide(SO₂), G = Agricultural chemicals, K = Ammonia(NH₃), Hg = Mercury, NO = Oxides of nitrogen, MB = Methyl bromide, AX = Low boiling point organic compounds(below 65 degC)

If inhalation risk above the TLV exists, wear approved dust respirator.

Use respirators with protection factors appropriate for the exposure level.

- Up to 5 X TLV, use valveless mask type; up to 10 X TLV, use 1/2 mask dust respirator
- Up to 50 X TLV, use full face dust respirator or demand type C air supplied respirator
- Up to 500 X TLV, use powered air-purifying dust respirator or a Type C pressure demand supplied-air respirator
- Over 500 X TLV wear full-face self-contained breathing apparatus with positive pressure mode or a combination respirator with a Type C positive pressure supplied-air full-face respirator and an auxiliary self-contained breathing apparatus operated in pressure demand or other positive pressure mode
- Respirators may be necessary when engineering and administrative controls do not adequately prevent exposures.
- The decision to use respiratory protection should be based on professional judgment that takes into account toxicity information, exposure measurement data, and frequency and likelihood of the worker's exposure - ensure users are not subject to high thermal loads which may result in heat stress or distress due to personal protective equipment (powered, positive flow, full face apparatus may be an option).
- Published occupational exposure limits, where they exist, will assist in determining the adequacy of the selected respiratory protection. These may be government mandated or vendor recommended.
- Certified respirators will be useful for protecting workers from inhalation of particulates when properly selected and fit tested as part of a complete respiratory protection program.
- Use approved positive flow mask if significant quantities of dust becomes airborne.
- Try to avoid creating dust conditions.

SECTION 9 PHYSICAL AND CHEMICAL PROPERTIES

Information on basic physical and chemical properties

Appearance	Off white to light brown crystallals with earthy smell; does not mix with water.		
Physical state	Divided Solid	Relative density (Water = 1)	1.7
Odour	Not Available	Partition coefficient n-octanol / water	Not Available
Odour threshold	Not Available	Auto-ignition temperature (°C)	Not Applicable
pH (as supplied)	Not Applicable	Decomposition temperature	Not Available
Melting point / freezing point (°C)	Not Available	Viscosity (cSt)	Not Applicable
Initial boiling point and boiling range (°C)	Not Available	Molecular weight (g/mol)	Not Applicable
Flash point (°C)	Not Applicable	Taste	Not Available
Evaporation rate	Not Applicable	Explosive properties	Not Available
Flammability	Not Applicable	Oxidising properties	Not Available
Upper Explosive Limit (%)	Not Applicable	Surface Tension (dyn/cm or mN/m)	Not Applicable
Lower Explosive Limit (%)	Not Applicable	Volatile Component (%vol)	Not Applicable
Vapour pressure (kPa)	Not Applicable	Gas group	Not Available
Solubility in water (g/L)	Immiscible	pH as a solution (1%)	Not Applicable
Vapour density (Air = 1)	Not Applicable	VOC g/L	Not Applicable

SECTION 10 STABILITY AND REACTIVITY

Reactivity	See section 7
Chemical stability	- Unstable in the presence of incompatible materials. - Product is considered stable. - Hazardous polymerisation will not occur.
Possibility of hazardous reactions	See section 7
Conditions to avoid	See section 7
Incompatible materials	See section 7
Hazardous decomposition products	See section 5

SECTION 11 TOXICOLOGICAL INFORMATION

Information on toxicological effects

Inhaled	Inhalation of dusts, generated by the material during the course of normal handling, may be damaging to the health of the individual.
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Continued...

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	Persons with impaired respiratory function, airway diseases and conditions such as emphysema or chronic bronchitis, may incur further disability if excessive concentrations of particulate are inhaled. If prior damage to the circulatory or nervous systems has occurred or if kidney damage has been sustained, proper screenings should be conducted on individuals who may be exposed to further risk if handling and use of the material result in excessive exposures. Effects on lungs are significantly enhanced in the presence of respirable particles. Overexposure to respirable dust may produce wheezing, coughing and breathing difficulties leading to or symptomatic of impaired respiratory function. Acute silicosis occurs under conditions of extremely high silica dust exposure particularly when the particle size of the dust is small. It differs greatly from classical silicosis both clinically and pathologically. The disease is rapidly progressive with diffuse pulmonary involvement developing only months after the initial exposure and causing deaths within 1 to 2 years. It is often complicated by an associated tuberculosis. The lungs of victims contain no classical silicotic nodules or only a few, microscopic abortive nodules, whereas the air spaces are diffusively filled and distended with silica-containing, lipoprotein paste in which degenerating and necrotic macrophages are sometimes discernible - the condition is sometimes described as alveolar lipoproteinosis. The uptake of silica particles by macrophages and lysosomal incorporation, is followed by rupture of the lysosomal membrane and release of lysosomal enzymes into cytoplasm of the macrophage. This causes the macrophage to be digested by its own enzymes and after lysis the free silica is released to be ingested by other macrophages thus continuing initiate collagen formation in the lung tissue producing the characteristic nodule found in classical (chronic) silicosis.
Ingestion	Accidental ingestion of the material may be damaging to the health of the individual. Not normally a hazard due to the physical form of product. The material is a physical irritant to the gastro-intestinal tract Phthalates (aromatic dicarboxylic acid esters), in general, exhibit low toxicity, partly because of poor absorption but mainly as a result of rapid metabolism in which the esters are saponified to phthalic acid (which is rapidly excreted) and the parent alcohol (which is subsequently metabolised). The pathology of these compounds seems to be related to the released alcohol and its biological effects. The rate of absorption of ingested phthalate esters is influenced by the content of dietary fat. Ingested phthalate esters may to a lesser degree be absorbed as the monoester derivatives or in the case of di(2-ethylhexyl)phthalate, as the diester. Cumulative toxicity of the phthalates has been observed on repeated administration. Both di-n-octyl phthalate and di(2-ethylhexyl)phthalate were found to have 22-28 times greater toxicity (based on LD50s) following repeated administration to animals. The liver has been shown to be the target organ affected by the phthalates. In general phthalates have induced liver enlargement; this increase in liver weight has been attributed to rapid cell division (hyperplasia) along with the detachment of cells (hypertrophy). The increase in liver weight caused by phthalates has been found to reverse to normal or even below normal levels on prolonged exposure. Exposure to phthalates, in general, has been found to be associated with a reduction in circulating cholesterol and serum triglyceride levels which accounted for a reduction in liver steroidogenesis. The phthalates also effect carbohydrate metabolism in the liver producing depleted glycogen electron transport inhibitors following interaction with mitochondria. Testicular atrophy produced in rats during feeding studies depends on the length and structure of the alcohol; in general the lower molecular weight esters produce the more severe effects. The toxicity of phthalic acid isomers decreases in the order o-phthalic acid, isophthalic acid and terephthalic acid. Phthalic acid is not metabolised but is excreted, unchanged, in the urine and faeces. Terephthalic acid appears to potentiate the biological effects of substances such as antibiotics, thiamine and sulfonamides.
Skin Contact	The material is not thought to produce adverse health effects or skin irritation following contact (as classified by EC Directives using animal models). Nevertheless, good hygiene practice requires that exposure be kept to a minimum and that suitable gloves be used in an occupational setting. Open cuts, abraded or irritated skin should not be exposed to this material Entry into the blood-stream through, for example, cuts, abrasions, puncture wounds or lesions, may produce systemic injury with harmful effects. Examine the skin prior to the use of the material and ensure that any external damage is suitably protected.
Eye	Limited evidence exists, or practical experience suggests, that the material may cause eye irritation in a substantial number of individuals and/or is expected to produce significant ocular lesions which are present twenty-four hours or more after instillation into the eye(s) of experimental animals. Repeated or prolonged eye contact may cause inflammation characterised by temporary redness (similar to windburn) of the conjunctiva (conjunctivitis); temporary impairment of vision and/or other transient eye damage/ulceration may occur.
Chronic	On the basis, primarily, of animal experiments, concern has been expressed that the material may produce carcinogenic or mutagenic effects; in respect of the available information, however, there presently exists inadequate data for making a satisfactory assessment. Harmful: danger of serious damage to health by prolonged exposure through inhalation. Serious damage (clear functional disturbance or morphological change which may have toxicological significance) is likely to be caused by repeated or prolonged exposure. As a rule the material produces, or contains a substance which produces severe lesions. Such damage may become apparent following direct application in subchronic (90 day) toxicity studies or following sub-acute (28 day) or chronic (two-year) toxicity tests. Limited evidence suggests that repeated or long-term occupational exposure may produce cumulative health effects involving organs or biochemical systems. Chronic symptoms produced by crystalline silicas included decreased vital lung capacity and chest infections. Lengthy exposure may cause silicosis a disabling form of pneumoconiosis which may lead to fibrosis, a scarring of the lining of the air sacs in the lung. Symptoms may appear 8 to 18 months after initial exposure. Smoking increases this risk. Classic silicosis is a chronic disease characterised by the formation of scattered, rounded or stellate silica-containing nodules of scar tissue in the lungs ranging from microscopic to 1.0 cm or more. The nodules isolate the inhaled silica particles and protect the surrounding normal and functioning tissue from continuing injury. Simple silicosis (in which the nodules are less than 1.0 cm in diameter) is generally asymptomatic but may be slowly progressive even in the absence of continuing exposure. Simple silicosis can develop in complicated silicosis (in which nodules are greater than 1.0 cm in diameter) and can produce disabilities including an associated tuberculous infection (which 50 years ago accounted for 75% of the deaths among silicotic workers). Crystalline silica deposited in the lungs causes epithelial and macrophage injury and activation. Crystalline silica translocates to the interstitium and the regional lymph nodes and cause the recruitment of inflammatory cells in a dose dependent manner. In humans, a large fraction of crystalline silica persists in the lungs. The question of potential carcinogenicity associated with chronic inhalation of crystalline silica remains equivocal with some studies supporting the proposition and others finding no significant association. The results of recent epidemiological studies suggest that lung cancer risk is elevated only in those patients with overt silicosis. A relatively large number of epidemiological studies have been undertaken and in some, increased risk gradients have been observed in relation to dose surrogates - cumulative exposure, duration of exposure, the presence of radiographically defined silicosis, and peak intensity exposure. Chronic inhalation in rats by single or repeated intratracheal instillation produced a significant increase in the incidences of adenocarcinomas and squamous cell carcinomas of the lung. Lifetime inhalation of crystalline silica (87% alpha-quartz) at 1 mg/m³ (74% respirable) by rats, produced an increase in animals with keratinising cystic squamous cell tumours, adenomas, adenocarcinomas, adenosquamous cell carcinomas, squamous cell carcinoma and nodular bronchiolar alveolar hyperplasia accompanied by extensive subpleural and peribronchiolar fibrosis, increased pulmonary collagen content, focal lipoproteinosis and macrophage infiltration. Thoracic and abdominal malignant lymphomas developed in rats after single intrapleural and intraperitoneal injection of suspensions of several types of quartz. Some studies show excess numbers of cases of scleroderma, connective tissue disorders, lupus, rheumatoid arthritis chronic kidney diseases, and end-stage kidney disease in workers NOTE: Some jurisdictions require health surveillance be conducted on workers occupationally exposed to silica, crystalline. Such surveillance should emphasise ▸ demography, occupational and medical history and health advice ▸ standardised respiratory function tests such as FEV1, FVC and FEV1/FVC ▸ standardised respiratory function tests such as FV1, FVC and FEV1/FVC ▸ chest X-ray, full size PA view ▸ records of personal exposure Overexposure to respirable dust may cause coughing, wheezing, difficulty in breathing and impaired lung function. Chronic symptoms may include decreased vital lung capacity, chest infections Repeated exposures, in an occupational setting, to high levels of fine- divided dusts may produce a condition known as pneumoconiosis which is the lodgement of any inhaled dusts in the lung irrespective of the effect. This is particularly true when a significant number of particles less than 0.5 microns (1/50,000 inch), are present. Lung shadows are seen in the X-ray. Symptoms of pneumoconiosis may include a progressive dry cough, shortness of breath on exertion

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(exertional dyspnea), increased chest expansion, weakness and weight loss. As the disease progresses the cough produces a stringy mucous, vital capacity decreases further and shortness of breath becomes more severe. Other signs or symptoms include altered breath sounds, diminished lung capacity, diminished oxygen uptake during exercise, emphysema and pneumothorax (air in lung cavity) as a rare complication.

Removing workers from possibility of further exposure to dust generally leads to halting the progress of the lung abnormalities. Where worker-exposure potential is high, periodic examinations with emphasis on lung dysfunctions should be undertaken

Dust inhalation over an extended number of years may produce pneumoconiosis. Pneumoconiosis is the accumulation of dusts in the lungs and the tissue reaction in its presence. It is further classified as being of noncollagenous or collagenous types. Noncollagenous pneumoconiosis, the benign form, is identified by minimal stromal reaction, consists mainly of reticulin fibres, an intact alveolar architecture and is potentially reversible.

- ▶ The use of ceramic fibres in the work place should be reviewed in the context of frequency of use and potential for exposure.
- ▶ In circumstances where the respiratory standards or excursion limits are approached, work areas should be designated by the use of ropes or other similar barriers and appropriate signs be utilised, where possible. This is especially true for all overhead work involving ceramic fibres.
- ▶ Employees not engaged in the ceramic fibre work should not be allowed within 3 metres of the work unless wearing suitable personal protective equipment (PPE).
- ▶ An example of the appropriate signage for the restricted area is:

CERAMIC FIBRE WORK AREA; FOLLOW SAFETY INSTRUCTIONS.

All installation and/ or removal practices should be designed to minimise the liberation of dusts or fibres.

For Installation:

- ▶ The ceramic fibre material should be kept in its storage container until installation is ready to proceed.
- ▶ Containers/ bags should only be opened within the designated work areas.
- ▶ Empty storage bags should be stored in waste containers along with waste material.

For Removal:

- ▶ Waste material should be wetted to prevent generation of dusts and placed in sealed containers to prevent dust/ fibre emissions.

Upon completion of installation/ removal:

- ▶ All excess material should be sealed in bags/ containers prior to removal from designated work area.
- ▶ Area should then be cleaned using an industrial vacuum cleaner.
- ▶ Any remaining contaminant material should be removed with minimum liberation of dusts/fibres.
- ▶ Wet mopping and wiping may be utilised in some instances when an industrial vacuum is not available.

DUROSAND K5	TOXICITY	IRRITATION
	Not Available	Not Available
silica crystalline - quartz	TOXICITY	IRRITATION
	Not Available	Not Available
diisononyl phthalate	TOXICITY	IRRITATION
	Oral (rat) LD50: >10000 mg/kg ^[2]	Not Available
glass fibres	TOXICITY	IRRITATION
	Not Available	Not Available

Legend:

1. Value obtained from Europe ECHA Registered Substances - Acute toxicity 2.* Value obtained from manufacturer's SDS. Unless otherwise specified data extracted from RTECS - Register of Toxic Effect of chemical Substances

SILICA CRYSTALLINE - QUARTZ	WARNING: For inhalation exposure <u>ONLY</u>: This substance has been classified by the IARC as Group 1: CARCINOGENIC TO HUMANS The International Agency for Research on Cancer (IARC) has classified occupational exposures to respirable (<5 um) crystalline silica as being carcinogenic to humans. This classification is based on what IARC considered sufficient evidence from epidemiological studies of humans for the carcinogenicity of inhaled silica in the forms of quartz and cristobalite. Crystalline silica is also known to cause silicosis, a non-cancerous lung disease. Intermittent exposure produces; focal fibrosis, (pneumoconiosis), cough, dyspnoea, liver tumours. * Millions of particles per cubic foot (based on impinger samples counted by light field techniques). NOTE : the physical nature of quartz in the product determines whether it is likely to present a chronic health problem. To be a hazard the material must enter the breathing zone as respirable particles.
	High Molecular Weight Phthalate Esters (HMWPEs) Category as defined by the Phthalate Esters Panel HPV Testing Group (2001) and OECD (2004). The HMWPE group includes chemically similar substances produced from alcohols having backbone carbon lengths of >= 7. Due to their similar chemical structure, category members are generally similar with respect to physicochemical, biological and toxicological properties or display an expected trend. Thus, read-across for toxicity endpoints is an appropriate approach to characterise selected endpoints for members of this category. In some cases the substances have ester side group constituents that span two subcategories (i.e., transitional and high molecular weight constituents). If the level of C4 to C6 constituents in the substance exceeded 10%, the substance was conservatively placed in the transitional subcategory. High molecular weight phthalates are used nearly exclusively as plasticisers of PVC. They are very poorly soluble in water, and have very low vapor pressure. The extant database demonstrates that these substances have few biological effects. A notable exception to this generalisation is that hepatocarcinogenicity has been observed for diisononyl phthalate (DINP). The hepatocarcinogenicity effects of DINP are by a mechanism (peroxisomal proliferation) to which rodents are particularly sensitive. However, it does not appear to be relevant to humans. The high molecular weight phthalates all demonstrate minimal acute toxicity, are not genotoxic, exhibit some liver and kidney effects at high doses, and are negative for reproductive and developmental effects. Further, the available data indicate that the toxicological activity of these molecules diminishes with increasing molecular weight. Studies on HMWPEs indicate that they are rapidly metabolised in the gastrointestinal tract to the corresponding monoester, absorbed and excreted primarily in the urine. Acute toxicity: The available data on phthalates spanning the carbon range from C8-C13 indicate that phthalate esters in the high molecular weight subcategory are not toxic by acute oral and dermal administration; LD50 values of all substances tested exceed the maximum amounts which can be administered to the animals. There are fewer data available on inhalation toxicity; only di-iso-nonyl phthalate (DINP) and di-iso-decyl phthalate (DIDP) have been tested. However, the phthalates in the high molecular weight subcategory have extremely low vapor pressures, and exposure by inhalation at potentially hazardous levels is not anticipated. Repeat dose toxicity: Several substances ranging from C8-C11 have been tested for repeated dose toxicity in studies ranging from 21 days to two years. Ditrdecyl phthalate (CAS 119-06-2) has been studied by the Japan Ministry of Health and Welfare (unpublished report) and data for this substance is used as read-across data for DTDP*. In addition results from repeat dose studies examining DINP (CAS 685 15-48-0) and DIDP (CAS 68515-49-1) are used as read across for the di C9-C11 phthalates (CAS 68515-43-5). The principal effects found are those associated with peroxisomal proliferation, including liver enlargement and induction of peroxisomal enzymes. As shown for example in a comparative study of liver effects, the strongest inducers of peroxisomal proliferation were DEHP, DINP, and DIDP with substances of shorter and longer ester side chains (e.g., 610P*, 711P*, and diundecyl phthalate - DUP)

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showing less pronounced effects. Thus, it is reasonable to conclude that other members of this subcategory would show effects similar to but not more pronounced than those associated with DINP and DIDP. It should also be noted that the relevance of these findings to human health is, at best, questionable. It has been shown that these effects are mediated through the peroxisome proliferation-activated receptor alpha (PPARα), and that levels of PPARα are much higher in rodents than humans. Thus, one would expect humans to be substantially less responsive than rodents to peroxisome proliferating agents. Empirical evidence supporting this postulation is provided by studies in primates in which repeated administration of DEHP and DINP had no effects on liver, kidney or testicular parameters.

In this regard it should also be noted that kidney enlargement is also commonly observed but normally without any pathological changes. There is a component of the kidney changes which is also PPARα-related. It has also been shown that in male rats, DINP induces an alpha 2u-globulin nephropathy which is male rat-specific but without relevance to humans. Thus, as was true for the liver changes, the relevance of the kidney changes to human health is also questionable. Finally, some of the lower molecular weight phthalates can induce testicular atrophy when administered to juvenile rats at high levels. However, the higher molecular weight phthalates including di-n-octyl phthalate (DnOP), DINP, DIDP, 610P, and 71 1P do not induce testicular atrophy. Further, the testis was not a target organ for DINP in either marmosets or cynomolgus monkeys. Thus, testicular atrophy is not an effect associated with phthalates in the high molecular weight subcategory.

Reproductive toxicity: Reproductive toxicity tests in rats have been carried out with DINP, DIDP, a linear C7-C9 phthalate (CAS 68515-41-3), a linear C9-C11 phthalate, and ditridecyl phthalate (Japan Ministry of Health and Welfare, unpublished report). None of these affected fertility or profoundly affected male reproductive development. A slight decrease in offspring viability was reported for both DIDP and ditridecyl phthalate at levels associated with maternal effects. DnOP was tested for effects on fertility in a continuous breeding protocol in mice, and, like the other members of this subcategory, did not reduce fertility. Thus, it can be concluded that the subcategory of high molecular weight phthalates do not affect fertility.

Developmental toxicity: Developmental toxicity tests in rats have been carried out with DINP; DIDP; C7-9 phthalate (CAS 68515-41-3); C9-11 phthalate (CAS 68515-43-5); and ditridecyl phthalate (CAS 119-06-2). None of the substances tested affected litter size, foetal survival or bodyweight, and none produced teratogenic effects. Increased frequencies of developmental variants including dilated renal pelvis, and supernumerary lumbar and cervical ribs were found at levels associated with maternal effects. The toxicological significance of these developmental variants is unclear. DnOP was not teratogenic in mice when tested at very high levels. Thus, it can be concluded that this subcategory of high molecular weight phthalates do not produce profound developmental effects in rodents.

Genotoxicity: The majority of the substances in the subcategory of high molecular weight phthalates have been tested for genetic activity in the Salmonella assay, and all were inactive. One large program covering many of these substances was carried out by the National Institute of Environmental Health Sciences. Similarly, a range of substances covering the majority of the carbon numbers in this subcategory were found to be inactive in mouse lymphoma tests Chromosomal Aberrations. Two representative members of the subcategory of high molecular weight phthalates (DINP and DIDP) have been tested for chromosomal mutation in the mouse micronucleus test, and both were inactive. Ditridecyl phthalate (CAS 119-06-2) induced neither structural chromosomal aberrations nor polyploidy in CHL cells up to the limit concentration of 4.75 mg/ml, in the absence or presence of an exogenous metabolic activation system (Japan Ministry of Health and Welfare, unpublished report). Further, all of the low molecular weight and transitional phthalates that have been tested were inactive.

*610P - mixed decyl, hexyl and octyl esters (CAS Rn: 68648-93-1)

*711P - C7,C11, branched and linear esters (CAS Rn: 111381-90-9)

* DTDP - di-C11-14, C13 rich ester (CAS 68515-47-9)

The material may produce peroxisome proliferation. Peroxisomes are single, membrane limited, cytoplasmic organelles that are found in the cells of animals, plants, fungi and protozoa. Peroxisome proliferators include certain hypolipidaemic drugs, phthalate ester plasticisers, industrial solvents, herbicides, food flavourings, leukotriene D4 antagonists and hormones. Numerous studies in rats and mice have demonstrated the hepatocarcinogenic effects of peroxisome proliferators, and these compounds have been unequivocally established as carcinogens. However it is generally conceded that compounds inducing proliferation in rats and mice have little, if any, effect on human liver except at very high doses or extreme conditions of exposure.

[Huls] The effects of DINP on fertility-related parameters such as reduced testosterone content and production and altered reproductive organ weights (with or without histopathologies) have been demonstrated in rats. Although quantitatively being less potent, DINP has exhibited adverse effects on the male reproductive system and sexual differentiation during development in a number of rodent studies (e.g. increased nipple retention, testicular pathology and decreased AGD/AGI in male offspring), which are components of the antiandrogenic pattern observed with diethylhexyl phthalate (DEHP) (a known reproductive toxicant). Foetal expression of genes involved in androgen synthesis such as StAR and Cyp11a were also reduced. There was also a report of increased gene expression levels of InsI3 (a foetal Leydig cell product critical for testis descent) that may infer the impaired testicular steroidogenesis following exposure to DINP at high doses (e.g. = 750 mg/kg bw/d). Reduced InsI3 was also reported in numerous studies with DEHP. Considering the chemical composition of DINP, which is represented as mixed phthalates with side-chains made up of 5–10% methylethylhexyl, limited evidence of the toxicological properties of transitional phthalates may be expected at high doses of DINP tested. The reduced pup weight was observed at approximately 100 mg/kg bw/d in both sexes, both in one- and two-generation reproductive studies in rats, in the absence of overt maternal toxicity. The pup weight reduction was also sustained and not considered solely related to low birth weight. In a post-natal toxicity study, reduced pup weight was also reduced at = 250 mg/kg bw/d. Therefore, this adverse effect of DINP is assessed as the most sensitive endpoint on offspring development. Overall, the available human data do not provide sufficient evidence for a causal relationship between exposure to DINP and adverse health effects in humans. There is also insufficient information to examine the mode of action of DINP on male reproductive tract development and sexual function in comparison with transitional phthalates. However, elements of the plausible mode of action for DINP effects on the male reproductive system, offspring growth and sexual differentiation are considered likely to be parallel in rats and humans if the exposure to DINP is high and within a critical window of development. Therefore, the effects observed in animal studies are regarded as relevant to a human risk assessment.

GLASS FIBRES

Borosilicate ingredients are insoluble, inert, and will not significantly penetrate the skin. The metal cations (such as lead, zinc, silver) are secure in the molecules and will not be bioavailable. Therefore, there would not be any systemic toxicity expected from dermal application or contact. These ingredients are not dermal irritants or sensitizers.

Borosilicate use in personal care products introduces the possibility of inhalation exposure. The particle size of borosilicate glasses was reported to range from 50 nm – 1000 µm with the largest portion being in the 50 – 300 µm range. The sizes of a substantial majority of the particles of these ingredients, as manufactured, are larger than the respirable range and/or aggregate and agglomerate to form much larger particles in formulation. These ingredients are reportedly used at concentrations up to 4% in cosmetic products that may be aerosolized and up to 97% in products that may become airborne. Around 95% – 99% of droplets/particles would not be respirable to any appreciable amount. Coupled with the small actual exposure in the breathing zone and the short exposure time, this information indicates that incidental inhalation would not be a significant route of exposure that might lead to local respiratory or systemic toxic effects.

Notably there is a lack of irritation or sensitization in tests of dermal exposure, no systemic toxicity at 5000 mg/kg, and the absence of genotoxicity in an Ames test for a supernatant of the related chemical calcium borosilicate. Borosilicate glasses are chemically inert and thus not systemically toxic.

The substance is classified by IARC as Group 3: NOT classifiable as to its carcinogenicity to humans.

For fibre glass wool: In October 2001, IARC classified fiber glass wool as Group 3, "not classifiable as to its carcinogenicity to humans." The 2001 decision was based on current human and animal research that shows no association between inhalation exposure to dust from fibre glass wool and the development of respiratory disease. This is a reversal of the IARC finding in 1987 of a Group 2B designation (possibly carcinogenic to humans) based on earlier studies in which animals were injected with large quantities of fiber glass. NTP and ACGIH have not yet reviewed the IARC reclassification or the most current fibre glass health research; at this time, both agencies continue to classify glass wool based on the earlier animal injection studies.

There is little evidence for acute toxicity after inhalation of rockwool/ slagwool/ glasswool/ mineral fibres (MMMF). Rockwool/glasswool administered by inhalation produced little pulmonary fibrosis in experimental animals. [IARC Monograph 43]

Animal studies with amorphous silica show that surviving rats rapidly recovered on removal from dust, the silica was largely eliminated and cellular nodules, perivascular infiltrations and emphysema were almost completely resolved [Patty's].

The dust has been associated with skin irritation due to the mechanical action of the fibres [CHEMINFO, Sax, ILO ENCYCLOPEDIA].

MMMF are manufactured to definite diameters and cannot split along their length rather they break across and form small particles not needles [FARIMA].

Asthma-like symptoms may continue for months or even years after exposure to the material ceases. This may be due to a non-allergenic condition known as reactive airways dysfunction syndrome (RADS) which can occur following exposure to high levels of highly irritating compound. Key criteria for the diagnosis of RADS include the absence of preceding respiratory disease, in a non-atopic individual, with abrupt onset of persistent asthma-like symptoms within minutes to hours of a documented exposure to the irritant. A reversible airflow pattern, on spirometry, with the presence of moderate to severe bronchial hyperreactivity

on methacholine challenge testing and the lack of minimal lymphocytic inflammation, without eosinophilia, have also been included in the criteria for diagnosis of RADS. RADS (or asthma) following an irritating inhalation is an infrequent disorder with rates related to the concentration of and duration of exposure to the irritating substance. Industrial bronchitis, on the other hand, is a disorder that occurs as result of exposure due to high concentrations of irritating substance (often particulate in nature) and is completely reversible after exposure ceases. The disorder is characterised by dyspnea, cough and mucus production. The substance is classified by IARC as Group 3:
NOT classifiable as to its carcinogenicity to humans.
Evidence of carcinogenicity may be inadequate or limited in animal testing.
The dust has been associated with skin irritation due to the mechanical action of the fibres [CHEMINFO, Sax, ILO ENCYCLOPAEDIA]. MMMF are manufactured to definite fibre diameters and cannot split along their length rather they break across and form small particles not needles [FARIMA].

Acute Toxicity	⊘	Carcinogenicity	✓
Skin Irritation/Corrosion	⊘	Reproductivity	⊘
Serious Eye Damage/Irritation	⊘	STOT - Single Exposure	⊘
Respiratory or Skin sensitisation	⊘	STOT - Repeated Exposure	✓
Mutagenicity	⊘	Aspiration Hazard	⊘

Legend: ✗ – Data available but does not fill the criteria for classification
✓ – Data available to make classification
⊘ – Data Not Available to make classification

SECTION 12 ECOLOGICAL INFORMATION

Toxicity

DUROSAND K5	ENDPOINT	TEST DURATION (HR)	SPECIES	VALUE	SOURCE
	Not Available	Not Available	Not Available	Not Available	Not Available
silica crystalline - quartz	ENDPOINT	TEST DURATION (HR)	SPECIES	VALUE	SOURCE
	Not Available	Not Available	Not Available	Not Available	Not Available
diisononyl phthalate	ENDPOINT	TEST DURATION (HR)	SPECIES	VALUE	SOURCE
	LC50	96	Fish	>0.1mg/L	2
	EC50	48	Crustacea	>0.06mg/L	2
	EC50	96	Algae or other aquatic plants	>2.8mg/L	1
	NOEC	504	Crustacea	0.0036mg/L	2
glass fibres	ENDPOINT	TEST DURATION (HR)	SPECIES	VALUE	SOURCE
	EC50	48	Crustacea	0.476mg/L	2
	NOEC	48	Crustacea	0.0032mg/L	2

Legend: *Extracted from 1. IUCLID Toxicity Data 2. Europe ECHA Registered Substances - Ecotoxicological Information - Aquatic Toxicity 3. EPIWIN Suite V3.12 (QSAR) - Aquatic Toxicity Data (Estimated) 4. US EPA, Ecotox database - Aquatic Toxicity Data 5. ECETOC Aquatic Hazard Assessment Data 6. NITE (Japan) - Bioconcentration Data 7. METI (Japan) - Bioconcentration Data 8. Vendor Data*

Harmful to aquatic organisms.
For silica:
The literature on the fate of silica in the environment concerns dissolved silica in the aquatic environment, irrespective of its origin (man-made or natural), or structure (crystalline or amorphous). Indeed, once released and dissolved into the environment no distinction can be made between the initial forms of silica. At normal environmental pH, dissolved silica exists exclusively as monosilicic acid [Si(OH)₄]. At pH 9.4 the solubility of amorphous silica is about 120 mg SiO₂/l. Quartz has a solubility of only 6 mg/l, but its rate of dissolution is so slow at ordinary temperature and pressure that the solubility of amorphous silica represents the upper limit of dissolved silica concentration in natural waters. Moreover, silicic acid is the bioavailable form for aquatic organisms and it plays an important role in the biogeochemical cycle of Si, particularly in the oceans.
In the oceans, the transfer of dissolved silica from the marine hydrosphere to the biosphere initiates the global biological silicon cycle. Marine organisms such as diatoms, silicoflagellates and radiolarians build up their skeletons by taking up silicic acid from seawater. After these organisms die, the biogenic silica accumulated in them partly dissolves. The portion of the biogenic silica that does not dissolve settles and ultimately reaches the sediment. The transformation of opal (amorphous biogenic silica) deposits in sediments through diagenetic processes allows silica to re-enter the geological cycle. Silica is labile between the water and sediment interface.
Ecotoxicity:
Fish LC50 (96 h): Brachydanio rerio >10000 mg/l; zebra fish >10000 mg/l
Daphnia magna EC50 (24 h): >1000 mg/l; LC50 924 h): >10000 mg/l
DO NOT discharge into sewer or waterways.

Persistence and degradability

Ingredient	Persistence: Water/Soil	Persistence: Air
diisononyl phthalate	HIGH	HIGH

Bioaccumulative potential

Ingredient	Bioaccumulation
diisononyl phthalate	LOW (BCF = 183.8)

Mobility in soil

Ingredient	Mobility
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diisononyl phthalate

LOW (KOC = 467200)

SECTION 13 DISPOSAL CONSIDERATIONS

Waste treatment methods

Product / Packaging disposal	- Containers may still present a chemical hazard/ danger when empty. - Return to supplier for reuse/ recycling if possible. Otherwise: - If container can not be cleaned sufficiently well to ensure that residuals do not remain or if the container cannot be used to store the same product, then puncture containers, to prevent re-use, and bury at an authorised landfill. - Where possible retain label warnings and SDS and observe all notices pertaining to the product. Legislation addressing waste disposal requirements may differ by country, state and/ or territory. Each user must refer to laws operating in their area. In some areas, certain wastes must be tracked. A Hierarchy of Controls seems to be common - the user should investigate: - Reduction - Reuse - Recycling - Disposal (if all else fails) This material may be recycled if unused, or if it has not been contaminated so as to make it unsuitable for its intended use. Shelf life considerations should also be applied in making decisions of this type. Note that properties of a material may change in use, and recycling or reuse may not always be appropriate. In most instances the supplier of the material should be consulted. DO NOT allow wash water from cleaning or process equipment to enter drains. - It may be necessary to collect all wash water for treatment before disposal. - In all cases disposal to sewer may be subject to local laws and regulations and these should be considered first. - Where in doubt contact the responsible authority. - Recycle wherever possible or consult manufacturer for recycling options. - Consult State Land Waste Management Authority for disposal. - Bury residue in an authorised landfill. - Recycle containers if possible, or dispose of in an authorised landfill.
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SECTION 14 TRANSPORT INFORMATION

Labels Required

Marine Pollutant	NO
HAZCHEM	Not Applicable

Land transport (ADG): NOT REGULATED FOR TRANSPORT OF DANGEROUS GOODS

Air transport (ICAO-IATA / DGR): NOT REGULATED FOR TRANSPORT OF DANGEROUS GOODS

Sea transport (IMDG-Code / GGVSee): NOT REGULATED FOR TRANSPORT OF DANGEROUS GOODS

Transport in bulk according to Annex II of MARPOL and the IBC code

Not Applicable

SECTION 15 REGULATORY INFORMATION

Safety, health and environmental regulations / legislation specific for the substance or mixture

SILICA CRYSTALLINE - QUARTZ(14808-60-7) IS FOUND ON THE FOLLOWING REGULATORY LISTS

Australia Exposure Standards

Australia Hazardous Substances Information System - Consolidated Lists

Australia Inventory of Chemical Substances (AICS)

International Agency for Research on Cancer (IARC) - Agents Classified by the IARC Monographs

DIISONONYL PHTHALATE(68515-48-0) IS FOUND ON THE FOLLOWING REGULATORY LISTS

Australia Inventory of Chemical Substances (AICS)

GLASS FIBRES(65997-17-3) IS FOUND ON THE FOLLOWING REGULATORY LISTS

Australia Exposure Standards

Australia Hazardous Substances Information System - Consolidated Lists

Australia Inventory of Chemical Substances (AICS)

National Inventory	Status
Australia - AICS	Y
Canada - DSL	Y
Canada - NDSSL	N (diisononyl phthalate; glass fibres; silica crystalline - quartz)
China - IECSC	Y
Europe - EINEC / ELINCS / NLP	Y
Japan - ENCS	N (diisononyl phthalate; glass fibres; silica crystalline - quartz)
Korea - KECI	Y
New Zealand - NZIoC	Y
Philippines - PICCS	Y
USA - TSCA	Y

Continued...

Legend:

Y = All ingredients are on the inventory

N = Not determined or one or more ingredients are not on the inventory and are not exempt from listing(see specific ingredients in brackets)

SECTION 16 OTHER INFORMATION**Other information****Ingredients with multiple cas numbers**

Name	CAS No
silica crystalline - quartz	14808-60-7, 122304-48-7, 122304-49-8, 12425-26-2, 1317-79-9, 70594-95-5, 87347-84-0, 308075-07-2
diisononyl phthalate	68515-48-0, 28553-12-0
glass fibres	65997-17-3, 94551-77-6, 1204320-21-7, 308066-97-9, 155775-82-9

Classification of the preparation and its individual components has drawn on official and authoritative sources as well as independent review by the Chemwatch Classification committee using available literature references.

The SDS is a Hazard Communication tool and should be used to assist in the Risk Assessment. Many factors determine whether the reported Hazards are Risks in the workplace or other settings. Risks may be determined by reference to Exposures Scenarios. Scale of use, frequency of use and current or available engineering controls must be considered.

Definitions and abbreviations

PC – TWA: Permissible Concentration-Time Weighted Average

PC – STEL: Permissible Concentration-Short Term Exposure Limit

IARC: International Agency for Research on Cancer

ACGIH: American Conference of Governmental Industrial Hygienists

STEL: Short Term Exposure Limit

TEEL: Temporary Emergency Exposure Limit,

IDLH: Immediately Dangerous to Life or Health Concentrations

OSF: Odour Safety Factor

NOAEL :No Observed Adverse Effect Level

LOAEL: Lowest Observed Adverse Effect Level

TLV: Threshold Limit Value

LOD: Limit Of Detection

OTV: Odour Threshold Value

BCF: BioConcentration Factors

BEI: Biological Exposure Index

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